TASK 1 / OBSERVE BEFORE REDESIGNING

# A room for shared care

The photographed room offers a useful starting point for collaboration, but it does not visibly provide a shared discussion area. This assessment separates what is visible from what still needs on-site verification.

## Functional for healthcare staff?  PARTLY

Visible: a hospital bed, overbed table, bedside cabinet and wall communication board. The open floor at the foot of the bed may help circulation. Concern: furniture beside the bed may compete with staff working space. Both-side access, equipment movement, door location, bathroom route and hand-hygiene facilities cannot be confirmed from this photo.

## Welcoming for families?  PARTLY

Visible: a recliner and a folded chair suggest that visitors are accommodated. However, a dedicated family work surface, belongings storage and a place to join bedside discussions are not evident. One photograph cannot establish whether overnight family accommodation is available.

## Warm and inviting?  PARTLY

The window, warm wall colors and wood-look finishes offer a less institutional appearance. The clinical bed and ceiling remain prominent. The photograph alone cannot establish comfort, lighting quality, noise levels or how patients and families experience the room.

## Promotes healing?  POTENTIALLY, NOT PROVEN

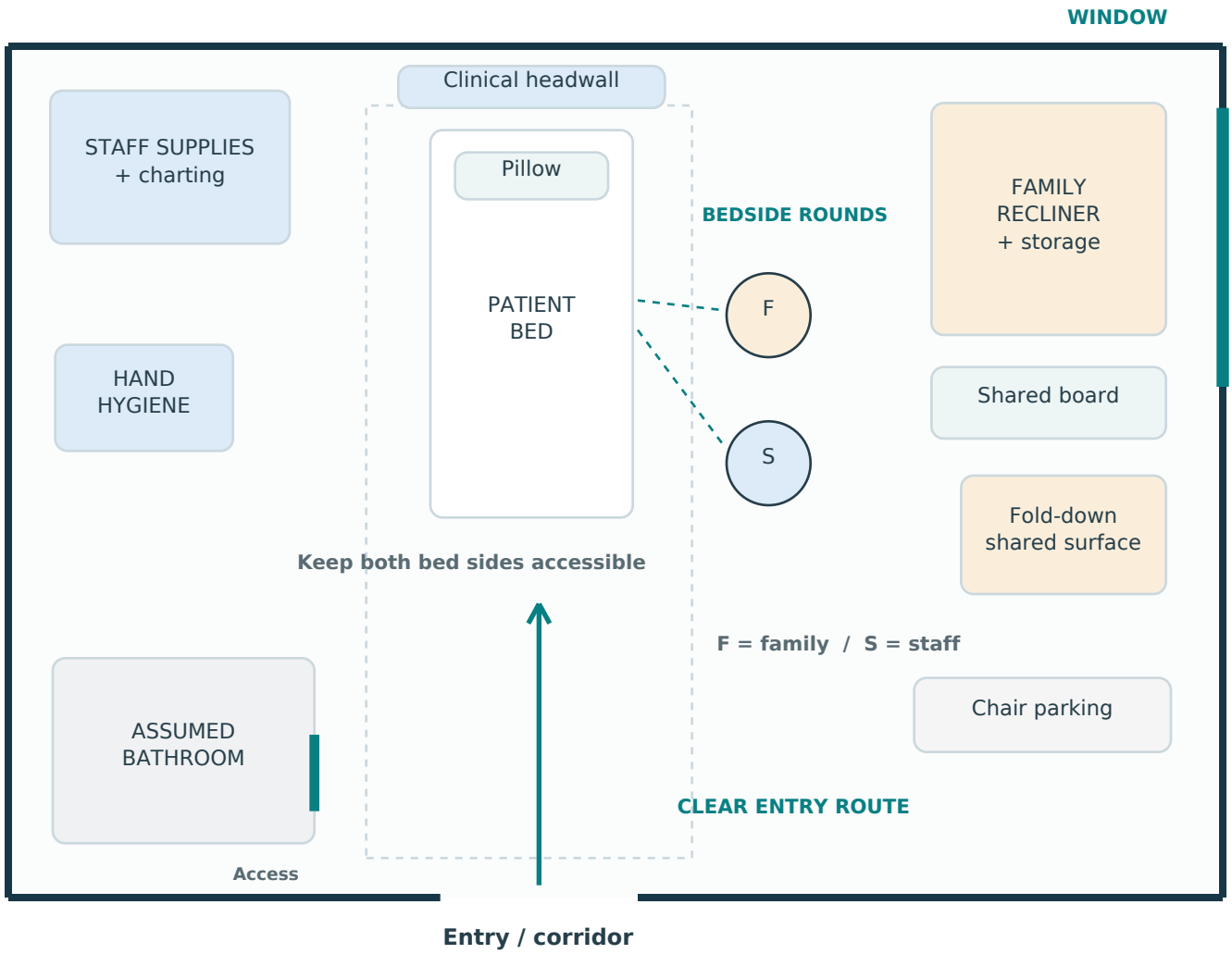
Daylight and a place for a support person may support comfort and rest. Actual effects on sleep, stress or recovery have not been measured. Privacy, nighttime lighting, acoustics and safe movement need further assessment.

**Design objective:** help the patient, their chosen support person and staff see one another, share information and discuss care without obstructing clinical work.
**PFCC basis:** dignity and respect, information sharing, participation and collaboration. Family involvement follows the patient's preferences.

TASK 2 / SKETCH A

# Bedside partnership

A shared conversation zone keeps the patient involved while preserving a staff access route.




Top-down concept. Not to scale. Door, bathroom and utilities are assumed.

**1. Participate at the bedside.** A movable family chair and staff stool support conversation at patient eye level. The recliner stays in the family zone; movable seats return to parking before hands-on care.

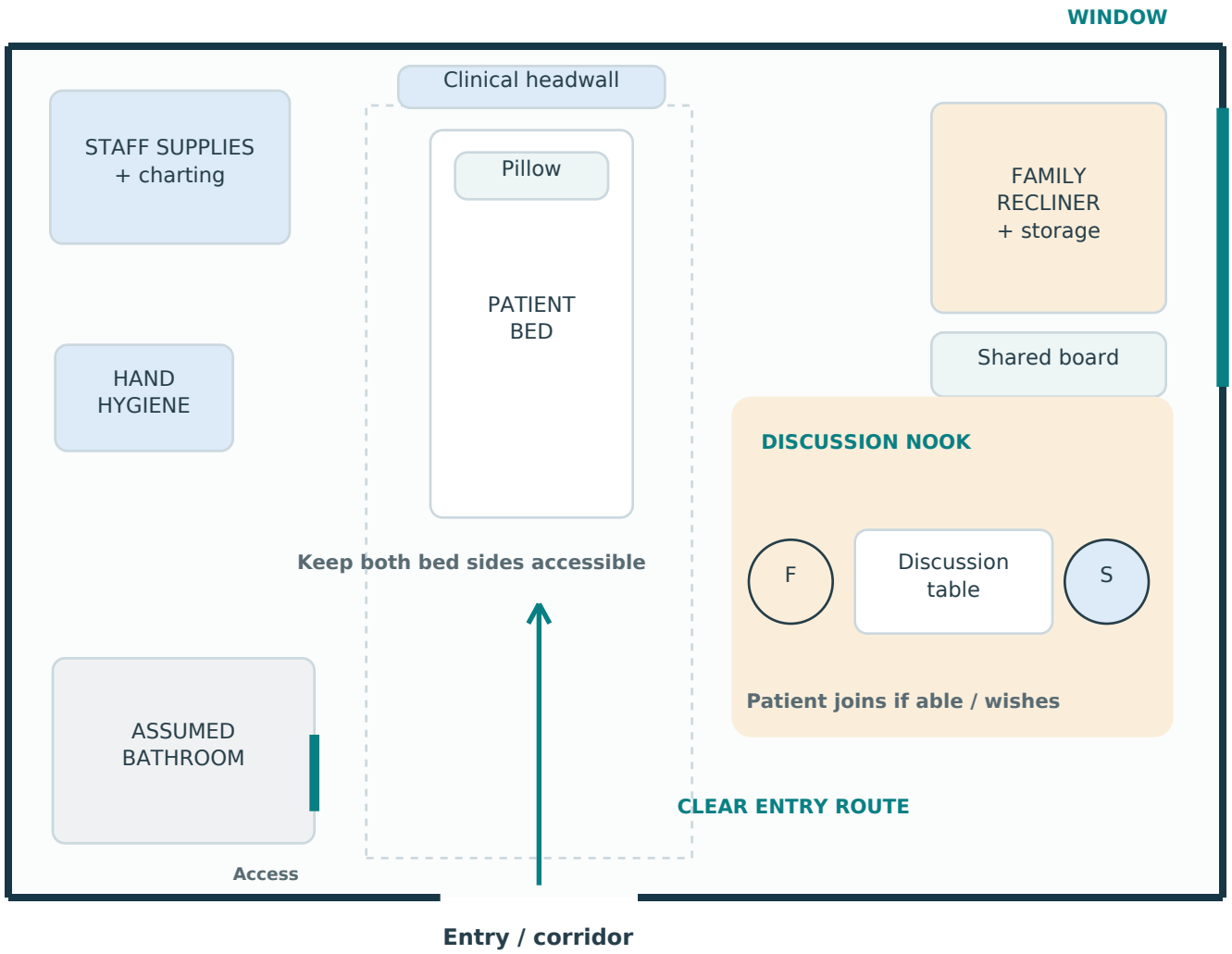
**2. Share the care plan.** An adjustable, legible board faces the patient and discussion seats. Use plain language, interpreter support when needed and only information the patient agrees to display.

**3. Protect clinical work.** Keep supplies and hand hygiene near staff entry. Store belongings off the floor. A fold-down surface supports family notes without adding a permanent table in the entry route. Test transfer and emergency access with furniture both in use and stowed.

TASK 2 / SKETCH B

# Family discussion nook

A separate table creates space for longer conversations and caregiver education.




Top-down concept. Not to scale. Door, bathroom and utilities are assumed.

**1. Create a dedicated meeting place.** A family member and clinician sit at a small table for discharge instructions, planning and note-taking. The patient joins when able and willing; otherwise the conversation moves to the bedside.

**2. Support extended presence.** A recliner and closed storage near the window provide a family base. Patient-controlled lighting and cleanable, warm finishes are proposed in both designs.

**3. Recognize the trade-off.** The nook consumes more usable floor area and may exclude a patient who remains in bed. It is not acoustically private merely because it is separate. Patient-inclusive discussion and clinical access must take priority over the table.

# Prototype Design A

Selection: bedside partnership. This is a low-fidelity concept ready for a walkthrough, not a built room.

## Functional for healthcare staff?  YES, CONDITIONALLY

It provides a direct approach from the entrance, a staff supply zone and a place to park movable seating. Real clearances, equipment positions, bathroom access and emergency workflows must be measured before calling the layout safe.

## Welcoming for families?  YES, BY DESIGN

A dedicated recliner, storage and shared surface acknowledge the support person's role. A movable discussion seat lets family join rounds with the patient's permission. Participation remains optional.

## Warm and inviting?  YES, AS AN INTENTION

Preserve daylight and add adjustable blinds, separate reading and clinical task lighting, warm cleanable finishes and optional calming artwork. Materials and furnishings require local cleaning and facilities review.

## Promotes healing?  PLAUSIBLE, TO BE TESTED

The proposal aims to support rest, dignity and understandable communication. It does not establish faster recovery or fewer clinical complications. Patient control over visitors, privacy and lighting is central.

## Why A rather than B?

Design A includes a bed-bound patient directly in the conversation and avoids a permanent discussion table. Design B is useful for longer teaching sessions, but requires more space and risks conversations happening away from the patient.

**Prototype setup and two states**
Use the page 2 plan as the paper prototype. Recreate the measured room with floor tape and movable furniture in a training space.
**Rounds:** bring the family chair and staff stool beside the patient; review the shared board together.
**Hands-on care:** park the seats, fold the surface and verify that staff can reach the bed and move equipment. Never park furniture on the bathroom or exit route.

TASK 4 / TEST BEFORE ADOPTING

# A practical testing plan

Proposed evaluation only. No participants have been recruited and no testing results are claimed.

**Participants:** invite 3 former adult patients (including a mobility-aid user), 3 family/support persons, 2 bedside nurses, 1 physician, 1 occupational or physical therapist, 1 infection-prevention specialist and 1 facilities specialist. Include varied language and communication needs; offer an interpreter and accessible feedback formats.

**Setting and method:** use a vacant training room, tape, mock equipment and furniture. Obtain voluntary consent; use fictional care information. Measure the real room first. Compare the existing layout with Design A using identical tasks, alternating which layout is tried first. Run 45-60 minute mixed-participant sessions; gather comments from patients and families separately as well as together.

### Bedside rounds

Patient, family and clinician review a fictional care plan. Ask: Could you see and hear everyone? Could you ask a question? Did the patient choose who participated?

### Teach-back

Explain a fictional discharge plan, then ask the patient and support person to explain it in their own words. Record misunderstandings and whether the board helped.

### Movement and urgent access

Simulate a bed transfer, mobility-aid route to the bathroom and an urgent staff entry with mock equipment. Observe furniture moves, collisions, trip risks and blocked access.

### Rest, privacy and cleaning

Try the recliner, blinds and lighting; check board visibility from the doorway. Ask where belongings go. Have staff assess whether proposed surfaces and furniture can be cleaned.

**Proposed acceptance targets:** zero observed blocked exit or essential care routes, collisions or trip hazards; all users can reach needed controls with their usual assistance; at least 80% of patient/family reviewers rate inclusion and comfort 4/5 or higher; at least 80% correctly explain the fictional plan. Track access time and furniture moves against the existing layout. These are pilot criteria, not regulatory standards.

**Revise and retest:** prioritize safety and patient exclusion issues, change one or two features, and repeat affected tasks. Obtain facilities, accessibility and infection-prevention review before any real-room change. A small simulation tests usability; it cannot prove clinical healing outcomes.